FROM THE

EDITOR-IN-CHIEF

Through the Covid-19 pandemic that inaugurated this decade, there was one refrain that we occasionally heard from health experts with their ear to the ground. That while the newly minted SARS-Cov-2 virus indeed caused a strange and often lethal form of flu, India could not afford to focus its periscope exclusively on its control. For, a much older affliction still stalked the land, silently exacting a consistently high toll: tuberculosis.

The spectre of TB not only continues to haunt India, it has been sharpening its knife-edge with a vengeance and coming for us in newer and more dangerous ways. Already we bear a big part of the global disease burden. Not only does India account for almost a quarter of the 10 million people afflicted with TB annually across the world, but of the yearly global toll of 1.4 million, India contributes 400,000 deaths or a little less than a third of them. And now, molecular analysis is disclosing to us a deadlier frontier of that war. The *Mycobacterium tuberculosis* develops gene mutations in response to common anti-TB drugs that render it a much more truculent enemy. The mutant form, known as Multi-Drug-Resistant (MDR) TB, and its even more murderous cousin—Extensively Drug-Resistant (XDR) TB—have seen an alarming rise of late. India has the world's highest burden of MDR-TB—its 119,000 cases account for 26 per cent of the global total.



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One of the challenges this situation poses is social. The reason why TB once tended to evoke a blasé response from the opinion-making classes had to do with class. TB was always seen as a disease of the poor. Conventional understanding was that the TB bacteria was more or less omnipresent—the majority of us were infected, but the deleterious effects would show only when our immunity systems were severely compromised. In India, that typically happened with malnutrition; globally, it was HIV patients who were particularly susceptible on that front. But while the cluster of proximate causes still bears a linear equation with poverty, TB is seen to have breached the class barrier. Among the reasons: a generalised lowering of immunity because of new urban ways of living, including poor dietary habits.

There are two related facts most people have not quite awoken to. One, that TB can be extremely infectious and goes through much the same pattern of asymptomatic carriers infecting others in conducive social situations. This is not to mention the 64 per cent TB-symptomatic individuals between 2019 and 2021 who did not seek healthcare, living with what they thought was common cough and not going beyond the usual over-the-counter drugs. Two, pulmonary TB—the prototypical sort that affects the lungs—is not the only type. This bacteria can take up residence in any part of the human body: liver, kidney, spine, even brain. And each makes for its own special kind of complication.

The other is the birth of mutations, and the human culpability on that front. The chief problem here is that the TB bacteria is an extremely tenacious one that can hold out against a long siege. Treatment is usually a long and torturous process, involving often months of the strictest multi-drug regimen. All a patient has to do is miss one dose somewhere—which most often happens when people let down their guards once they feel better—and the bacteria adapts to that particular combo-drug fusillade. The only option then is to move to even more harsh medication.

Senior Associate Editor Sonali Acharjee's cover story this week maps out the new dangers of MDR and XDR TB. Architect Debshree Lokhande of Pune presents before us the unnerving set of complications that can arise for patients. By the time it was discovered that her TB strain was able to withstand three of four drugs in the standard drug ensemble, she had lost one lung. The treatment moved up to the then-new experimental drug bedaquiline, and it took three excruciating years of physical struggle and severe side-effects—loss of hearing, for one—before she was back on her feet. This is the ambient danger for everyone: this mutant bacteria will be among a whole cohort that are at large, all of them fortified through the same trap-escape-mutation cycle. This class of drug-resistant bacteria looms in our midst as an amorphous superfoe that needs only a slight lapse in our defences to get ahead of us. So the goalpost, by definition, is a shifting one: the next patient is perhaps being infected by a newer mutant. And some 56 per cent of them are undiagnosed and 64 per cent untreated: they are moving around in public spaces amidst us.

In an endeavour to bolster its defences, India has set itself a stern challenge. In 2018, Prime Minister Narendra Modi announced that the country would strive to eradicate tuberculosis by 2025, five years ahead of the United Nations target of 2030. Fortunately, the system was alive to the dangers of taking the eyes off the ball on account of Covid-19, and put in place a Rapid Response Plan that ensured the National TB Elimination Programme (NTEP) stayed on course through the turbulence caused by the Covid first and second waves. The total number of TB patients notified during those years—19,33,381 in 2021 as compared to 16,28,161 in 2020—demonstrated the keenness of this vigil.

More recently, while inaugurating the One World TB Summit in Varanasi this March, Modi reiterated the government's resolve to eliminate the scourge in the next two years. As many as 250,000 gram panchayats will be brought in as the advance guard in this grim battle. The overall numbers suggest the 2025 deadline will be difficult to keep, but the scaling up of the fortress walls is by itself salutary. And because India is a hotspot, on its success will hinge the larger objective of global TB eradication. So, in the next two years India should redouble its efforts to ensure that it's rid of this modern-day plague.



(Aroon Purie)